

Colon & Colorectal Cancer — FDA Approved Drugs Intelligence

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Situation Summary

Biomarker: 16 require biomarker testing
Immunotherapy: 3 immunotherapy agents
Targeted: 16 biomarker-directed agents
Breakthrough: 16 drugs scored ≥ 4

FDA Approved Drug Timeline

FDA-approved colorectal cancer drugs ranked by clinical significance. Drugs requiring biomarker testing are marked with the required biomarker. Click any drug name to view the full FDA approval record.

HIGH **IMMUNOTHERAPY** **REQUIRES: MSI-H/dMMR** FDA APPROVED 20241227

5

Nivolumab (Opdivo)

Nivolumab, marketed as Opdivo, is approved for the treatment of colorectal cancer in patients with microsatellite instability-high (MSI-H) or mismatch repair-deficient (dMMR) tumors, representing a specific line of therapy for this patient population. This drug functions as an immunotherapy by blocking the programmed death receptor-1 (PD-1), which helps enhance the immune system's ability to fight cancer, making the biomarker status critical for identifying eligible patients. Before considering this drug, patients should discuss their MSI-H/dMMR status with their healthcare provider, as this determines the appropriateness of nivolumab for their treatment. Opdivo is part of a class of PD-1 inhibitors, which includes other options that may have different indications or mechanisms of action.

HIGH **RET-TARGETED** **REQUIRES: RET fusion** FDA APPROVED 20240410

5

Selpercatinib (Retevmo)

Selpercatinib, marketed as Retevmo, is approved for the treatment of adult patients with locally advanced or metastatic non-small cell lung cancer (NSCLC) with a RET gene fusion, as well as for adult and pediatric patients aged 2 years and older with advanced or metastatic medullary thyroid cancer (MTC) with a RET mutation. This drug functions as a RET inhibitor, targeting specific genetic alterations that drive tumor growth, making the identification of the RET fusion biomarker essential for treatment eligibility. Before considering Retevmo, patients should discuss the necessity of undergoing an FDA-approved test to confirm the presence of the RET fusion or mutation. Retevmo is structurally distinct from other kinase inhibitors, providing targeted therapy options for patients with specific genetic profiles.

HIGH **NTRK-TARGETED** **REQUIRES: NTRK fusion** FDA APPROVED 20231020

5

Entrectinib (Rozlytrek)

Rozlytrek (entrectinib) is approved for the treatment of adult and pediatric patients with solid tumors that have a neurotrophic tyrosine receptor kinase (NTRK) gene fusion, as well as for adult patients with ROS1-positive metastatic non-small cell lung cancer (NSCLC). This drug functions as a TRK inhibitor, targeting tumors with specific genetic alterations, making the identification of the NTRK fusion biomarker crucial for its use. Before considering Rozlytrek, patients should be aware of the need for genetic testing to confirm the presence of the NTRK fusion and the absence of

known acquired resistance mutations. Rozlytrek is structurally distinct from other kinase inhibitors, which may influence its efficacy and safety profile in treating these specific cancer types.

HIGH

KRAS-TARGETED

REQUIRES: KRAS G12C

FDA APPROVED 20221212

5

Adagrasib (Krazati)

Krazati (adagrasib) is approved for the treatment of adult patients with KRAS G12C-mutated locally advanced or metastatic colorectal cancer (CRC) in combination with cetuximab, representing a targeted therapy option for patients who have received at least one prior systemic therapy. As a KRAS G12C inhibitor, Krazati specifically targets the KRAS G12C mutation, which is a critical biomarker that can influence treatment decisions and outcomes. Before considering Krazati, patients should confirm the presence of the KRAS G12C mutation through an FDA-approved test, as this biomarker is essential for the drug's efficacy. Krazati is structurally distinct from other therapies in the same class, focusing specifically on the KRAS G12C mutation.

HIGH

KRAS-TARGETED

REQUIRES: KRAS G12C

FDA APPROVED 20210528

5

Sotorasib (Lumakras)

Sotorasib, marketed as Lumakras, is approved for the treatment of adult patients with KRAS G12C-mutated locally advanced or metastatic non-small cell lung cancer (NSCLC) who have received at least one prior systemic therapy. This drug functions as a KRAS G12C inhibitor, targeting a specific mutation that plays a critical role in the growth and spread of cancer cells. Before considering Sotorasib, patients should confirm the presence of the KRAS G12C mutation through an FDA-approved test, as this biomarker is essential for the drug's efficacy. Lumakras is distinct in its class as it specifically targets the KRAS G12C mutation, which is not addressed by other available therapies.

HIGH

HER2-TARGETED

REQUIRES: HER2+

FDA APPROVED 20200417

5

Tucatinib (Tukysa)

Tukysa (generic name: Tucatinib) is approved for the treatment of adult patients with RAS wild-type HER2-positive unresectable or metastatic colorectal cancer, representing a targeted therapy option in this setting. As a HER2 inhibitor, Tukysa works by blocking the HER2 protein, which is overexpressed in some cancer cells, making the biomarker HER2 status critical for determining eligibility for treatment. Before considering Tukysa, patients should discuss their HER2 status and any previous treatments with their healthcare provider to ensure it aligns with the drug's indications. Tukysa is structurally distinct from other HER2 inhibitors, providing a targeted approach specifically for HER2-positive tumors.

HIGH

HER2-TARGETED

REQUIRES: HER2+

FDA APPROVED 20191220

5

Trastuzumab deruxtecan (Enhertu)

Enhertu (trastuzumab deruxtecan) is approved for the treatment of HER2-positive metastatic breast cancer, serving as a first-line therapy in combination with pertuzumab or as a monotherapy for patients who have received prior treatment. This drug functions as an antibody-drug conjugate (ADC) that targets the HER2 protein, which is significant as the presence of HER2+ biomarkers indicates a specific type of breast cancer that may respond to this therapy. Before considering Enhertu, patients should discuss their HER2 status and previous treatments with their healthcare provider to determine if this drug is appropriate for their situation. Enhertu is structurally distinct from other HER2-targeted therapies, offering a unique mechanism of action through its conjugation with a topoisomerase inhibitor.

HIGH

NTRK-TARGETED

REQUIRES: NTRK fusion

FDA APPROVED 20181126

5

Larotrectinib (Vitrakvi)

Vitrakvi (larotrectinib) is approved for the treatment of adult and pediatric patients with solid tumors that have an NTRK gene fusion, representing a targeted therapy option for those with metastatic disease or where surgery may lead to significant complications. As a TRK inhibitor, Vitrakvi specifically targets tumors with NTRK fusions, making the identification of this biomarker crucial for treatment eligibility. Before considering Vitrakvi, patients should discuss with their healthcare provider whether their tumor has the NTRK fusion and if they meet the criteria for this therapy. Vitrakvi is distinct in its targeted approach compared to other treatments that may not focus on specific genetic alterations.

HIGH

BRAF-TARGETED

REQUIRES: BRAF V600E

FDA APPROVED 20180627

5

Encorafenib (Braftovi)

Encorafenib, marketed as Braftovi, is approved for the treatment of adult patients with metastatic colorectal cancer (mCRC) in combination with cetuximab and fluorouracil-based chemotherapy. This drug functions as a BRAF inhibitor, specifically targeting the BRAF V600E mutation, which is crucial for determining eligibility for treatment. Before considering Braftovi, patients should be aware that they must have the BRAF V600E mutation confirmed by an FDA-authorized test. Structurally, Braftovi belongs to the same class as other BRAF inhibitors, but its specific combination with cetuximab and chemotherapy distinguishes its use in colorectal cancer.

HIGH

BRAF-TARGETED

REQUIRES: BRAF V600E

FDA APPROVED 20180627

5

Binimetinib (Mektovi)

Mektovi (binimetinib) is approved for use in combination with encorafenib for the treatment of patients with unresectable or metastatic melanoma and adult patients with metastatic non-small cell lung cancer (NSCLC), both with a BRAF V600E or V600K mutation. As a MEK inhibitor, Mektovi targets the MAPK signaling pathway, which is often activated in tumors with these specific BRAF mutations, making the biomarker testing essential for identifying eligible patients. Before considering Mektovi, patients should discuss potential side effects and the importance of confirming their BRAF mutation status through an FDA-approved test. Mektovi is structurally distinct from other kinase inhibitors, providing a targeted approach to treatment in specific patient populations.

HIGH

IMMUNOTHERAPY

REQUIRES: MSI-H/dMMR

FDA APPROVED 20140904

5

Pembrolizumab (Keytruda)

Pembrolizumab, marketed as Keytruda, is an FDA-approved immunotherapy for the treatment of colorectal cancer in patients with microsatellite instability-high (MSI-H) or mismatch repair deficient (dMMR) tumors. It functions as a programmed death receptor-1 (PD-1) blocking antibody, which helps to enhance the immune response against cancer cells, making the biomarker status critical for identifying eligible patients. Before considering this drug, patients should discuss their MSI-H/dMMR status with their healthcare provider, as this determines the appropriateness of pembrolizumab for their treatment. Keytruda is part of a class of immunotherapies that target the PD-1 pathway, similar to other agents in this category.

HIGH

HER2-TARGETED

REQUIRES: HER2+

FDA APPROVED 20120608

5

Pertuzumab (Perjeta)

Pertuzumab, marketed as Perjeta, is approved for use in combination with trastuzumab and docetaxel for adults with HER2-positive metastatic breast cancer who have not received prior anti-HER2 therapy or chemotherapy for metastatic disease. It acts as a HER2/neu receptor antagonist, targeting the HER2 biomarker, which is crucial for identifying patients who may benefit from this treatment. Before considering Pertuzumab, patients should discuss their HER2 status and any previous treatments with their healthcare provider to ensure it is an appropriate option for their specific situation. Pertuzumab is structurally similar to trastuzumab, as both are designed to target the HER2 receptor, but they have different mechanisms of action that can enhance treatment efficacy when used together.

HIGH

IMMUNOTHERAPY

REQUIRES: MSI-H/dMMR

FDA APPROVED 20110325

5

Ipilimumab (Yervoy)

Ipilimumab, marketed as Yervoy, is an FDA-approved immunotherapy for the treatment of unresectable or metastatic melanoma, representing a first-line therapy option. It functions as a CTLA-4-blocking antibody, which enhances the immune response against cancer cells, and is indicated for patients with the biomarker MSI-H/dMMR. Before considering Yervoy, patients should discuss potential side effects and the importance of biomarker testing with their healthcare provider. Yervoy is structurally distinct from other immunotherapies, such as PD-1 inhibitors, which target different pathways in the immune response.

HIGH

ANTI-EGFR

REQUIRES: RAS wild-type

FDA APPROVED 20060927

5

Panitumumab (Vectibix)

Panitumumab, marketed as Vectibix, is an FDA-approved anti-EGFR therapy indicated for the treatment of adult patients with wild-type RAS metastatic colorectal cancer (mCRC). It works by targeting the epidermal growth factor receptor (EGFR), and the requirement for RAS wild-type status is crucial as it determines the drug's effectiveness in this patient population. Before considering Vectibix, patients should be aware of the necessity for testing to confirm their RAS status.

and discuss potential side effects with their healthcare provider. Structurally, Vectibix is an antibody that differs from other anti-EGFR agents, which may influence treatment decisions based on individual patient profiles.

HIGH

ANTI-EGFR

REQUIRES: RAS wild-type

FDA APPROVED 20040212

5

Cetuximab (Erbix)

Cetuximab, marketed as Erbitux, is an FDA-approved anti-EGFR therapy indicated for the treatment of colorectal cancer in patients with RAS wild-type tumors, typically used in the second-line setting or beyond. This drug works by targeting the epidermal growth factor receptor (EGFR), which plays a crucial role in tumor growth and progression, making the biomarker status important for determining eligibility. Before considering cetuximab, patients should discuss their RAS status with their healthcare provider, as the presence of RAS mutations can affect treatment efficacy. Cetuximab is structurally distinct from other anti-EGFR agents, which may influence its use in specific patient populations.

MEDIUM

ANTI-EGFR

FDA APPROVED 20231108

3

Fruquintinib (Fruzaqla)

This drug is FDA-approved for the treatment of adult patients with metastatic colorectal cancer (mCRC) who have been previously treated with specific chemotherapy and anti-VEGF therapy, and it is classified as a VEGFR inhibitor with no biomarker requirement.

MEDIUM

CHEMOTHERAPY

FDA APPROVED 20150922

2

Trifluridine (Lonsurf)

This drug is FDA-approved for the treatment of metastatic colorectal cancer in adult patients who have been previously treated with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, with no biomarker requirement, and belongs to the chemotherapy drug class.

MEDIUM

ANTI-EGFR

FDA APPROVED 20140421

2

Ramucirumab (Cyramza)

This drug is FDA-approved for the treatment of advanced or metastatic gastric or gastro-esophageal junction adenocarcinoma with disease progression on or after prior fluoropyrimidine- or platinum-containing chemotherapy, with no biomarker requirement, and belongs to the anti-VEGFR2 drug class.

MEDIUM

MULTI-KINASE

FDA APPROVED 20120927

2

Regorafenib (Stivarga)

This drug is FDA-approved for the treatment of metastatic colorectal cancer in adult patients who have been previously treated with specific chemotherapy and therapies, with no biomarker requirement, and belongs to the multi-kinase drug class.

MEDIUM

ANTI-VEGF

FDA APPROVED 20120803

2

Aflibercept (Zaltrap)

This drug is FDA-approved for the treatment of metastatic colorectal cancer (mCRC) that is resistant to or has progressed following an oxaliplatin-containing regimen, with no biomarker requirement, and belongs to the anti-VEGF drug class.

MEDIUM

CHEMOTHERAPY

FDA APPROVED 20050131

2

Oxaliplatin (Eloxatin)

This drug is FDA-approved for the treatment of colorectal cancer, with no biomarker requirement, and belongs to the chemotherapy drug class.

MEDIUM**ANTI-VEGF**

FDA APPROVED 20040226

 2**Bevacizumab** (Avastin)

This drug is FDA-approved for the treatment of metastatic colorectal cancer, with no biomarker requirement, and belongs to the anti-VEGF drug class.

MEDIUM**CHEMOTHERAPY**

FDA APPROVED 19980430

 2**Capecitabine** (Xeloda)

This drug is FDA-approved for the treatment of colorectal cancer, including adjuvant treatment of Stage III colon cancer and perioperative treatment of locally advanced rectal cancer, with no biomarker requirement, and belongs to the chemotherapy drug class.

MEDIUM**CHEMOTHERAPY**

FDA APPROVED 19960614

 2**Irinotecan** (Camptosar)

This drug is FDA-approved for the treatment of metastatic carcinoma of the colon or rectum, with no biomarker requirement, and belongs to the chemotherapy drug class.

MEDIUM**CHEMOTHERAPY** 2**Fluorouracil** (5-FU)

This drug is FDA-approved for treating colorectal cancer, has no biomarker requirement, and belongs to the chemotherapy drug class.

Appendix A — Patient Guide to Approved Drugs

For patients, caregivers, and family members. Always discuss treatment options with your oncologist — which drug is right for you depends on your tumor's molecular profile, stage, and prior treatment history.

Why biomarker testing matters before treatment

Many FDA-approved colorectal cancer drugs only work — or are only approved — for tumors with a specific genetic mutation. Getting complete biomarker testing at diagnosis (RAS, BRAF, MSI/MMR, HER2, NTRK) ensures you know which of these drugs you may qualify for. Testing should happen before your first treatment begins.

Nivolumab (Opdivo)

IMMUNOTHERAPY · Requires: MSI-H/dMMR

Nivolumab, marketed as Opdivo, is approved for the treatment of colorectal cancer in patients with microsatellite instability-high (MSI-H) or mismatch repair-deficient (dMMR) tumors, representing a specific line of therapy for this patient population. This drug functions as an immunotherapy by blocking the programmed death receptor-1 (PD-1), which helps enhance the immune system's ability to fight cancer, making the biomarker status critical for identifying eligible patients. Before considering this drug, patients should discuss their MSI-H/dMMR status with their healthcare provider, as this determines the appropriateness of nivolumab for their treatment. Opdivo is part of a class of PD-1 inhibitors, which includes other options that may have different indications or mechanisms of action.

Selpercatinib (Retevmo)

RET-TARGETED · Requires: RET fusion

Selpercatinib, marketed as Retevmo, is approved for the treatment of adult patients with locally advanced or metastatic non-small cell lung cancer (NSCLC) with a RET gene fusion, as well as for adult and pediatric patients aged 2 years and older with advanced or metastatic medullary thyroid cancer (MTC) with a RET mutation. This drug functions as a RET inhibitor, targeting specific genetic alterations that drive tumor growth, making the identification of the RET fusion biomarker essential for treatment eligibility. Before considering Retevmo, patients should discuss the necessity of undergoing an FDA-approved test to confirm the presence of the RET fusion or mutation. Retevmo is structurally distinct from other kinase inhibitors, providing targeted therapy options for patients with specific genetic profiles.

Entrectinib (Rozlytrek)

NTRK-TARGETED · Requires: NTRK fusion

Rozlytrek (entrectinib) is approved for the treatment of adult and pediatric patients with solid tumors that have a neurotrophic tyrosine receptor kinase (NTRK) gene fusion, as well as for adult patients with ROS1-positive metastatic non-small cell lung cancer (NSCLC). This drug functions as a TRK inhibitor, targeting tumors with specific genetic alterations, making the identification of the NTRK fusion biomarker crucial for its use. Before considering Rozlytrek, patients should be aware of the need for genetic testing to confirm the presence of the NTRK fusion and the absence of known acquired resistance mutations. Rozlytrek is structurally distinct from other kinase inhibitors, which may influence its efficacy and safety profile in treating these specific cancer types.

Adagrasib (Krazati)

KRAS-TARGETED · Requires: KRAS G12C

Krazati (adagrasib) is approved for the treatment of adult patients with KRAS G12C-mutated locally advanced or metastatic colorectal cancer (CRC) in combination with cetuximab, representing a targeted therapy option for patients who have received at least one prior systemic therapy. As a KRAS G12C inhibitor, Krazati specifically targets the KRAS G12C mutation, which is a critical biomarker that can influence treatment decisions and outcomes. Before considering Krazati, patients should confirm the presence of the KRAS G12C mutation through an FDA-approved test, as this biomarker is essential for the drug's efficacy. Krazati is structurally distinct from other therapies in the same class, focusing specifically on the KRAS G12C mutation.

Tucatinib (Tukysa)

HER2-TARGETED · Requires: HER2+

Tukysa (generic name: Tucatinib) is approved for the treatment of adult patients with RAS wild-type HER2-positive unresectable or metastatic colorectal cancer, representing a targeted therapy option in this setting. As a HER2 inhibitor, Tukysa works by blocking the HER2 protein, which is overexpressed in some cancer cells, making the biomarker HER2 status critical for determining eligibility for treatment. Before considering Tukysa, patients should discuss their HER2 status and any previous treatments with their healthcare provider to ensure it aligns with the drug's indications. Tukysa is structurally distinct from other HER2 inhibitors, providing a targeted approach specifically for HER2-positive tumors.

Encorafenib (Braftovi)

BRAF-TARGETED · Requires: BRAF V600E

Encorafenib, marketed as Braftovi, is approved for the treatment of adult patients with metastatic colorectal cancer (mCRC) in combination with cetuximab and fluorouracil-based chemotherapy. This drug functions as a BRAF inhibitor, specifically targeting the BRAF V600E mutation, which is crucial for determining eligibility for treatment. Before considering Braftovi, patients should

be aware that they must have the BRAF V600E mutation confirmed by an FDA-authorized test. Structurally, Braftovi belongs to the same class as other BRAF inhibitors, but its specific combination with cetuximab and chemotherapy distinguishes its use in colorectal cancer.

Panitumumab (Vectibix)

ANTI-EGFR · **Requires: RAS wild-type**

Panitumumab, marketed as Vectibix, is an FDA-approved anti-EGFR therapy indicated for the treatment of adult patients with wild-type RAS metastatic colorectal cancer (mCRC). It works by targeting the epidermal growth factor receptor (EGFR), and the requirement for RAS wild-type status is crucial as it determines the drug's effectiveness in this patient population. Before considering Vectibix, patients should be aware of the necessity for testing to confirm their RAS status and discuss potential side effects with their healthcare provider. Structurally, Vectibix is an antibody that differs from other anti-EGFR agents, which may influence treatment decisions based on individual patient profiles.

Trifluridine (Lonsurf)

CHEMOTHERAPY ·

This drug is FDA-approved for the treatment of metastatic colorectal cancer in adult patients who have been previously treated with fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, with no biomarker requirement, and belongs to the chemotherapy drug class.

Regorafenib (Stivarga)

MULTI-KINASE ·

This drug is FDA-approved for the treatment of metastatic colorectal cancer in adult patients who have been previously treated with specific chemotherapy and therapies, with no biomarker requirement, and belongs to the multi-kinase drug class.

Aflibercept (Zaltrap)

ANTI-VEGF ·

This drug is FDA-approved for the treatment of metastatic colorectal cancer (mCRC) that is resistant to or has progressed following an oxaliplatin-containing regimen, with no biomarker requirement, and belongs to the anti-VEGF drug class.

Questions to ask your oncologist

"Has my tumor been tested for all the biomarkers that have an FDA-approved treatment?"

"Which of these approved drugs apply to my specific mutation profile?"

"What line of therapy is each drug approved for — first-line, second-line, or later?"

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This is not medical advice. Always speak with your oncologist before making any treatment decisions.

Appendix B — CRC Drug Reference Guide

Biomarker-Directed Therapies

MSI-H/dMMR — Pembrolizumab (Keytruda), Nivolumab (Opdivo) + Ipilimumab (Yervoy)

BRAF V600E — Encorafenib (Braftovi) + Cetuximab or Binimetinib

HER2+ — Tucatinib (Tukysa), Trastuzumab deruxtecan (Enhertu), Pertuzumab + Trastuzumab

NTRK fusion — Larotrectinib (Vitrakvi), Entrectinib (Rozlytrek)

KRAS G12C — Sotorasib (Lumakras), Adagrasib (Krazati)

RAS wild-type — Cetuximab (Erbix), Panitumumab (Vectibix) [anti-EGFR agents]

Chemotherapy Backbones

FOLFOX — Fluorouracil + Leucovorin + Oxaliplatin

FOLFIRI — Fluorouracil + Leucovorin + Irinotecan

FOLFOXIRI — All three combined; used in high-volume metastatic disease

Capecitabine (Xeloda) — Oral fluorouracil alternative

Trifluridine/tipiracil (Lonsurf) — Later-line oral chemotherapy

Anti-Angiogenic Agents

Bevacizumab (Avastin) — Anti-VEGF; added to FOLFOX or FOLFIRI in first-line

Ramucirumab (Cyramza) — Anti-VEGFR2; second-line with FOLFIRI

Aflibercept/Ziv-aflibercept (Zaltrap) — Anti-VEGF; second-line

Fruquintinib (Fruzaqla) — VEGFR inhibitor; later-line, no biomarker required

Regorafenib (Stivarga) — Multi-kinase inhibitor; later-line

Key principle

Most biomarker-directed drugs require confirmed mutation status before use. RAS and BRAF testing is required before starting anti-EGFR agents. MSI/MMR testing determines immunotherapy eligibility. Always test before starting first-line therapy — not after.

Key Resources

FDA Drug Approvals (accessdata.fda.gov) · NCCN Guidelines ([nccn.org](https://www.nccn.org)) · Colorectal Cancer Alliance ([ccalliance.org](https://www.ccalliance.org)) · Fight Colorectal Cancer ([fightcrc.org](https://www.fightcrc.org))

Colon & Colorectal Cancer — FDA Approved Drugs Intelligence

About This Report

This report is generated automatically from publicly available medical databases including PubMed, ClinicalTrials.gov, and the FDA Drug@FDA database. Entries are classified using deterministic rule-based logic and scored by clinical significance. AI-generated summaries are grounded exclusively in the source data provided.

Limitations

This report does not represent a complete picture of all published research or approved treatments. Database coverage, feed availability, and relevance filtering affect what appears. New drug approvals and trial status changes may not be reflected immediately. Always verify current information at the original source before making clinical decisions.

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